OPERATION: Excision of primary nasal pterygium with superior conjunctival-limbal autograft and fibrin glue fixation, right eye.

PREOPERATIVE DIAGNOSIS: Symptomatic, visually threatening primary nasal pterygium extending 3.5 mm across the cornea toward the visual axis, right eye.

SURGICAL TECHNIQUE: Following subconjunctival anesthesia with 2% lidocaine with epinephrine injected directly into the pterygium body, an eyelid speculum was placed. The apex of the pterygium on the cornea was grasped with 0.12 mm forceps. A Beaver 64 blade was used to perform meticulous dissection of the pterygium head off the corneal surface, establishing a smooth Bowman's membrane plane.

The abnormal subconjunctival fibrovascular tissue (pterygium body) was dissected free from the overlying conjunctiva and the underlying Tenon's capsule back to the medial rectus muscle insertion. The diseased tissue was excised, ensuring the medial rectus muscle was fully visualized and unharmed. A diamond burr was applied to polish the corneal bed until completely smooth and free of residual stromal scar.

The bare sclera defect was measured with calipers (6 x 5 mm). Attention was directed to the superotemporal bulbar conjunctiva. Calipers were used to mark an oversized 6.5 x 5.5 mm thin conjunctival-limbal graft, taking care to avoid Tenon's tissue. The graft was carefully harvested, noting the limbal orientation. The graft was transferred to the nasal bed, maintaining correct limbal-to-limbal and epithelial orientation.

Tisseel fibrin sealant (two-component thrombin and fibrinogen) was placed onto the bare sclera bed, and the autograft was smoothed over the defect with gentle compression for three minutes. Adhesion was verified to be completely stable without sutures. A therapeutic bandage contact lens was placed over the cornea. Topical moxifloxacin and prednisolone acetate drops were administered.